

Knee and Lower Leg Conditions Disability Benefits Questionnaire

SYNTHETIC REPLICA for software testing. Structure follows VA Form 21-0960M-9. All data fictional.

|                     |                                                                   |
|---------------------|-------------------------------------------------------------------|
| Form number         | 21-0960M-9                                                        |
| Veteran name        | Leilani K. Ahuna                                                  |
| Date of birth       | 01/16/1990                                                        |
| SSN                 | 900-45-6789                                                       |
| Date of examination | 08/12/2026                                                        |
| Examiner            | Dr. A. Fictional, MD                                              |
| Knee examined       | <input checked="" type="radio"/> Right <input type="radio"/> Left |

Section 1. Diagnosis

|              |                                               |
|--------------|-----------------------------------------------|
| Diagnosis #1 | Right knee strain with chondromalacia patella |
| ICD code     | M22.41                                        |

Section 2. Medical history

|                                    |                                                                                                                                          |
|------------------------------------|------------------------------------------------------------------------------------------------------------------------------------------|
| History (onset, course, treatment) | Veteran reports right knee pain beginning in service after a fall during a field exercise in 2011, treated conservatively. Symptoms have |
|------------------------------------|------------------------------------------------------------------------------------------------------------------------------------------|

Section 3. Flare-ups

|                                                           |                                                               |
|-----------------------------------------------------------|---------------------------------------------------------------|
| Does the Veteran report flare-ups?                        | <input checked="" type="radio"/> Yes <input type="radio"/> No |
| Describe (frequency, duration, severity, functional loss) |                                                               |

Section 4. Functional loss or impairment

|                                         |                                                               |
|-----------------------------------------|---------------------------------------------------------------|
| Functional loss or impairment reported? | <input checked="" type="radio"/> Yes <input type="radio"/> No |
| Describe                                | Difficulty with prolonged standing, squatting, and stairs.    |

## Section 5. Initial range of motion (ROM) measurements

Normal knee ROM: flexion 0 to 140 degrees; extension 140 to 0 degrees (38 CFR 4.71, Plate II).

Flexion (0-140), degrees

Extension (140-0), degrees short of 0

Is ROM abnormal or outside normal range?

☒ Yes ☐ No

If abnormal, does ROM contribute to functional loss?

☒ Yes ☐ No ☐ Not addressed

Is pain noted on examination (on ROM)?

☒ Yes ☐ No

Does the pain cause functional loss?

☒ Yes ☐ No ☐ Not addressed

Evidence of pain with weight-bearing?

☒ Yes ☐ No ☐ Not tested

Evidence of pain in nonweight-bearing?

☒ Yes ☐ No ☐ Not tested

Evidence of pain on passive ROM testing?

☒ Yes ☐ No ☐ Not tested

If not tested, explain

Objective evidence of localized tenderness or pain on palpation?

☒ Yes ☐ No

Objective evidence of crepitus?

☒ Yes ☐ No

Is the opposing joint undamaged?

☒ Yes ☐ No

Opposing joint flexion, degrees

Opposing joint extension, degrees

## Section 6. Repetitive-use testing

Was repetitive-use testing (3 repetitions) performed?

☒ Yes ☐ No

If not performed, explain

Flexion after 3 repetitions, degrees

Extension after 3 repetitions, degrees

Additional loss of function or ROM after repetitions?

☒ Yes ☐ No

## Section 7. Flare-ups and repeated use over time

Is the examination being conducted during a flare-up?

☐ Yes ☐ No

Do pain, weakness, fatigability or incoordination significantly limit functional ability during flare-ups?

☐ Yes ☐ No ☐ Unable to say without...

Estimated flexion during flare-ups, degrees

Estimated extension during flare-ups, degrees

If unable to estimate, explain

*Contributing factors of disability (check all that apply):*

- ☒ Pain on movement
- ☐ Weakened movement
- ☒ Excess fatigability
- ☐ Incoordination
- ☒ Less movement than normal
- ☐ Swelling
- ☐ Atrophy of disuse
- ☐ Instability of station

## Section 9. Muscle strength testing

Knee flexion strength (0/5 - 5/5)

Knee extension strength (0/5 - 5/5)

Muscle atrophy?

☐ Yes ☐ No

## Section 10. Ankylosis

Ankylosis of the knee?

☐ Yes ☐ No

**Section 11. Joint stability tests**

Anterior instability (Lachman test)

☒ Normal ☐ 1+ ☐ 2+ ☐ 3+  
☐ Not tested

Posterior instability (posterior drawer test)

☒ Normal ☐ 1+ ☐ 2+ ☐ 3+  
☐ Not tested

Medial instability (valgus pressure)

☒ Normal ☐ 1+ ☐ 2+ ☐ 3+  
☐ Not tested

Lateral instability (varus pressure)

☒ Normal ☐ 1+ ☐ 2+ ☐ 3+  
☐ Not tested

History of recurrent subluxation

☒ None ☐ Slight ☐ Moderate ☐ Severe
**Section 18. Diagnostic testing**

Imaging studies performed and reviewed?

☒ Yes ☐ No

Degenerative or traumatic arthritis documented?

☒ Yes ☐ No
**Section 19. Functional impact**

Impact on ability to work

The knee condition limits prolonged standing and walking; the Veteran can perform sedentary work with position changes.

**Section 20. Remarks**

Remarks, if any

Additional loss of 15 degrees of flexion after three repetitions due to pain and fatigability.

Medical opinion (if requested)

Was a medical opinion requested?

☒ Yes ☐ No

Opinion

☒ At least as likely as... ☐ Less likely than not ...  
☐ Cannot be determined ...

Rationale

The current right knee condition is at least as likely as not related to the documented in-service fall because the service treatment reco